

**Reading, Reflection, Critical Analysis, and Synthesis Paper**

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**Abstract**

The purpose of this paper is to reflect upon, analyze critically, and show a significant synthesis of the scholarly material concerning the DSM-5 and psychological assessment, case conceptualization, and treatment found in Dailey et al. (2014) and Antony & Barlow (2020). Selected chapters were read by the author and interesting and important topics and areas of proper client care were noted. These areas of interest and important topics are discussed and expanded upon in the following document.

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The ethical and effective care for those seeking mental health care by counselors starts with a proper understanding of assessment, case conceptualization, and treatment planning. As a counselor, my pledge to my clients is to "do no harm" and provide care that benefits them (ACA, 2014). In my master's level education, I was introduced to and asked to begin interfacing with these tools. However, admittedly, my intentionality in doing so has been lacking. It is challenging and developmental to take up these two texts and read them. Reading these two texts with current, former, and future clients in mind offered a new perspective. I understand more fully (though not perfectly) how proper case management can increase the likelihood of success in therapy. I begin with reflections, critical analysis, and synthesis of Dailey et al. Chapter One.

#### **Reflections from Dailey et al., Chapter One**

Chapter One is an overview of the changes and revisions to the newest form of the Diagnostic and Statistics Manual of Mental Disorders (DSM). The content of this chapter is normally not interesting to me. However, this chapter has a motivating section on the why of diagnosis.

The definition given at the outset of the chapter is helpful and clarifying. I agree that counseling is first and foremost a professional relationship. I also agree that a counselor's role is to help clients meet their goals. For many years I was a pastor. As a pastor people have an expectation of guidance and direction. More of the people I met with in this capacity were asking direct questions of me to figure out what they needed. Where this type of relationship is important and our world needs people who are willing to speak truth, I find I prefer the type of relationship that being a counselor affords me. I prefer to be the kind of truth teller that points people back to humanity's source for strength and safety in God the Trinity. Rather than being

the source for them, I enjoy the ability to help them stay connected to the vine that Jesus tells us about in John 15 (English Standard Version Bible, 2001).

This definition of counseling is succinct enough to be applicable to most counselors and most counseling settings. There is a definitive target of empowering others. And yet, there is enough freedom to define that praxis for oneself. This definition is certainly workable as I look forward to educating and supervising emerging counselors.

A second why the authors give for acquiring diagnostic skills is to satisfy the Council for Accreditation of Counseling and Related Educational Programs (CACREP) requirement of collaborating and communicating with other health care providers. As I read this section it became apparent to me that I have some work to do in this area. In the past four years of active professional counseling, I have managed to stay isolated apart from a few critical cases. A year ago, my organization brought on another full-time licensed counselor. Soon after we began supervising local Master level emerging counselors. Both necessary steps of becoming a counseling center are in the back of my mind as I process this motivating reason for becoming a more proficient diagnostician. If I cannot speak the common language of counselors and mental health providers, I will lack in providing the care my clients and the clinic's clients deserve. If I cannot interact with the industry standard of care, how will I educate and mentor the future of the field? My lack of experience and knowledge in this area does not dissuade me from moving forward. On the contrary, this lack encourages me to seek the degree of Ph.D. with more resolve.

Even with the resolve to move forward with this degree and become a more proficient diagnostician, I cannot help but agree with the authors' two points concerning resistance to DSM use. Not wanting to stigmatize and "medicalize" my clients is in direct relation to the reason I like the ACA definition of counseling. These two overly medical and overly pragmatic views of

clients tend to diminish the relational aspect that is proven to be so powerful in the field of counseling. Throughout the history of the development of counseling theory, our understanding of the relationship continues to point to the possibility of healing and change based solely on a strong therapeutic bond. Many recent studies like one from Cozolino in 2006 demonstrate the ability of an interpersonal relationship in counseling to heal and change the traumatized brain. I will need to work through the dissonance of professionalism in diagnostic formulation and the practical application of my theory of change as I navigate this degree. With that navigation and other aspects of clinical mental health, I turn to chapter two for further insight.

### **Reflections from Dailey et al., Chapter Two**

The history of the DSM provided by the authors is quite intriguing. I often find that a historical view of any system or organization is helpful to me in finding connections and shared passion. With the history of this psychological tool in hand, I make two observations. First, it is not lost on me that the direct and intentional work of those who have gone before me is foundational to our field in general. And second, the openness of the field to adjust the tools that help us provide good care is equally as necessary. Changing not only the criteria for diagnosing over the years but also changing the philosophy of diagnosis shows the impetus for the creation of the tools was more altruistic than I first thought.

Adding to this understanding from historical accounts, I feel less alienated in my view of the DSM. It is encouraging to read that, throughout history, others have challenged the ubiquitous philosophy of certain theoretical structures. DSM-type disorders and understandings of the human condition are only one way of viewing humanity, our need for healing, and our idea of how to bring that healing. The fact that psychologists and counselors have worked together (albeit less at times than needed) to continue to transition the DSM from a one-size-fits-

all resource to a part of the overall picture of how we see humans. Recognizing the inclusions, integrations, and adaption of the biopsychosocial approach to diagnosis is heartening. And yet, my worldview brings a slightly different perspective on inclusion.

Over the years the sciences have begun to see inclusion as the opposite of bias. And where I am not one to stand for bias, it is not necessarily the opposite of inclusion. Attending to cultures, races, ethnicities, belief systems, and gender definitions that are something other than white, middle-American, Christian male is not inclusion, it's human. We can elevate the idea of pursuing an understanding of humans that are different from ourselves to religious status. As a counselor in the twenty-first century, I am proud that my predecessors who edited and compiled the DSM-IV-TR made it a priority to assess and mandate diagnoses based on cultural nuances. As a counselor in the twenty-first century, I am saddened that we feel obligated to make the case for those humans that are (by no choice of their own) born and raised not like me. It would be my hope to be able to speak to the oneness of humanity from a biblical inclusion model in the future. To instill in emerging counselors the ordinariness of recognizing difference as an opportunity to learn and grow rather than shaming our field for not being one of the others. A critical-biblical view of diversity, equity, and inclusion integrated into psychological diagnosis could bring us together as a unified front against mental health disorders. Currently, I see us moving apart based on our differences regardless of the efforts of our organizations to address the cultural outcry to be seen and heard.

In addition, the authors point to be cautious not to lean on diagnosis so strongly that we end up causing more harm than good is a valid argument. Cultural diagnosis, gender diagnosis, or genetic pathology trends in the DSM 5 are much improved over the generations before. However, we must remain vigilant as a professional body, not to lose sight of the dangers of

medicating without relational therapy. We must also not lose sight of the dangers of allowing popular psychology to influence our care. We can use the diagnostic resources to aid in the overall plan of care for each client as an individual. If we keep the DSM 5 and the accompanying assessments and treatment recommendations as a whole, I believe we will do just this. Taking the diagnoses as an end in themselves will bring only negative impact on individuals' mental and spiritual health.

In moving from general to specific, the following sections are my reflections and conclusions on specific diagnostic criteria and disorders. Each of these criteria and disorders is important to me and brought insight to me due to the regularity with which I encounter clients who fall under or could fall under these diagnoses. I begin with a closer look at General Anxiety Disorder.

### **Reflections from Dailey et al., Chapter Five and Antony & Barlow, Chapter Eight**

Worry. A word that in psychology and related arenas carries one connotation but for those who have been raised with an awareness of Christian Scripture carries a completely different connotation. The context of that word matters more than those outside the Christian culture understand. Defining generalized anxiety disorder (GAD) and the related disorders from the DSM 5 with an emphasis on worry has the effect of triggering some followers of Christ to toxic faith messages. Worry is seen as sin (missing the target God placed in front of us). Worry is seen in very black and white terms in many faith messages. The toxicity of the messages around worry and the triggering effect, makes me both thankful the disorder was not changed to General Worry Disorder and gives me pause when considering assigning this diagnosis. If we are called as counselors to be sensitive to the culture of our clients, this is a knowledge base of specific interest to me. I find myself using the word rumination, or reassurance seeking when helping

clients understand their anxiety. For those who have been told to just stop it, the validation and normalization of the intrusive worry is calming. I find that many times a willingness to acknowledge the human condition (including mine in use of self) can disarm the hyperaroused state of clients.

It was helpful to read from both texts that GAD is just as often as not related to Major Depressive Disorder (MDD). I have anecdotal data that supports the hypothesis that GAD and MDD are comorbid conditions. A large majority of my clientele present with both upon intake. To see the evidence from research emphasizes the need to assess for both and the need to concurrently treat for both.

I am also grateful to read that both disorders respond well to counseling therapies in general. I am not surprised that cognitive therapies are listed in both texts as the treatment of choice. I do hope that in the near future we can research more modalities in closer proximity to my preferred theories of Interpersonal and Attachment. The more I am exposed to Dan Siegel (2012) and the idea that healing through relationships is possible as we repair out of a place of mindfulness, the more I long for data to express this truth and for diagnostic criteria to reflect what he claims. I am encouraged to read from both books that some form of relaxation and mindfulness should be included with Cognitive Behavioral Therapy and other behavioral therapies. My longing is not completely without voice.

As I read these chapters I am struck with the intentionality and effort a clinician needs to put into staying informed. Not just with the diagnosis, assessment, and treatment of GAD, but with all the disorders. Finding the time to manage one's schedule to educate and reeducate oneself is crucial and stressful. Particularly staying abreast of the newest and best assessments



delineated in Antony and Barlow. This truly is a lifestyle and not just a job. Possibly even Obsessive Compulsive, which is my next section of disorder.

### **Reflections from Dailey et al., Chapter Six and Antony & Barlow, Chapter Nine**

These chapters were of current concern to me considering two clients that I am seeking to serve. These two young people have been diagnosed with obsessive compulsive disorder (OCD) by other mental health professionals. I have felt underqualified to serve them and treat their condition since intake about a year ago. Several aspects of these chapters clarified my ability to treat these clients from my theoretical perspective.

Reading that OCD is a response to stress helped make sense of the onset of this coping mechanism for anxiety in both clients. Each of them had a life event early in development that I saw as a direct stimulus in their neurobiological response through OCD. I also see the need to avoid the triggers related to their sexual trauma and religious trauma respectively. Their compulsions are so closely tied to the obsession of not letting the same trauma happen again it's both clarifying and delicate; like removing a tumor from the heart. I disagree, from a conceptual basis, that stress and exterior factors are not precipitating events in the onset of OCD because of these two clients (I recognize that my small sample size and lack of measures is a problem. However, the similarities in even this small sample size are too compelling to overlook.). External compulsions such as hand washing, and pacing started too contemporaneous with the traumatic events to not see causation. I understand that genetic factors have been empirically substantiated in recent studies, so I am not disagreeing with the DSM 5 etiology. I am simply making explicit the need to differentiate post-traumatic stress (PTS) from OCD.

Ironically one of these clients is open to and increasingly medicated with anxiolytic and antidepressant medications and the other is resistant to medication altogether. Not surprisingly

the one with religious trauma is resistant to medication. While I simply support and recommend on these issues generally, it is curious how little the medication seems to change the therapeutic response. One caveat to that statement was when the client who continues to increase medications was changing medications. During that time our ability to see progress diminished significantly. I attribute this to the effects of the anxiolytic on panic. When the client was in a constant state of hyperarousal due to past trauma, therapy was reduced to calming.

On that note, I agree with differentiating PTS from OCD in the DSM 5, to a point. With the neurobiological reaction of compulsions being so closely tied to the trauma triggers, perhaps the sections of trauma and anxiety should have stayed connected to the section of OCD. And yet, the symptomology is specific enough to have separate concepts of each constellation of disorders makes clinical sense. I am thinking particularly about the irresistible nature of the compulsions as a delineating factor. The next section will discuss the surplus research related to PTS that surely aided in the decision to separate the disorders.

### **Reflections from Dailey et al., Chapter Seven and Antony & Barlow, Chapter Ten**

The first facet of trauma and stressor-related disorders I am struck by is the sheer volume of research that has gone into the study of post-traumatic stress. The degree of diagnosis and assessment given in the DSM 5 points to large sections of population and studies that have been utilized to extract such voluminous information. I am a bit divided on the value of the resources both financially and in time. I do not fail to see the damage traumatic events have caused our generations of humans. From World War I and the atrocities of trench warfare to Vietnam and 911, we are a traumatized people (and this just in the most recent generations). And yet, the most populous clientele is apparently those dealing with smaller traumas (no less important to diagnose and treat) and other disorders such as anxiety and depression, not to mention grief. So,

my point is not that we should not study trauma and the effects thereof on the human body and brain. My point is that we are charged with finding the funding and impetus to study any condition that affects our human brothers and sisters equally. I assume that the U.S. Government's attempts to mitigate the damage trauma has caused our military (having a great deal of respect for and personal relationship with these individuals, I tread lightly here) has added resources to this effort. I also imagine that our only homeland was of 911 and the trauma it brought to all of us has contributed to the slough of research on PTS. And, again, I am not arguing against this effort. I am simply expressing our need for a great deal more research and differentiated treatment modalities across the board of mental disorders.

The fact that we have such specific data on children is both encouraging and saddening as well. To note that many of our innocent children have been affected so deeply is not okay. Many of these people having been wounded in childhood will suffer the consequences through a lifetime of searching for healing. I am not skilled at childhood disorders and treatments. It makes me sad that I freeze when anyone under the age of 16 sits on my couch. I long to develop the skills I need to help these little ones heal. My generation caused much of their wounding. I hope to be a part of my generation who brings healing to them. So, surly, we can find ways to translate the studies done with adolescents and children with PTSD to children with other trauma and stressor-related disorders like reactive attachment disorder and adjustment disorder. I may not be the guy who can do all of this, I only have so much time and energy. But this is why I want to be a part of educating and mentoring the next generation of emerging counselors. By producing and releasing competent counselors with passions in diverse areas and facets of the field, we can make headway in multiple dimensions of mental health.

Having said all of this, one of my concerns with trauma and stressor-related disorders is the prevalence of self-diagnosis and social-media dissemination of symptoms. Many authors, speakers and personalities have given personal testimony and views of the nature of trauma. Much of this is helpful and, from what I can tell, in keeping with the DSM 5 criteria for a potentially traumatizing event and related symptoms. However, trauma is a buzz word. In my circles this has become dividing. This particular reflection doesn't have space to delineate the division, but I feel the pressure of the division as a licensed professional counselor to educate and communicate to bring unity. Our early adolescents are demanding treatment over events that could be addressed by parents and supporting adults. Romantic break-ups and perceived scholastic failures are all events that can traumatize, but do not, in my opinion, fit the criteria for one of these disorders. In the final section of this paper, I will discuss my specific implications for the field of professional counseling where it pertains to this matter. For now, we turn to the diagnosis of personality disorders.

### **Reflections from Dailey et al., Chapter 16**

Personality disorders are scary. Fortunately, one of the statements I gravitated toward from this chapter was that I am not, nor do I need to worry about being the primary caregiver for individuals diagnosed with a personality disorder. It is reassuring to read that I will more than likely not be in a position to assess and diagnose these individuals either. The length of the chapter and the amount of detail given to this class of disorder also speaks to the typical counselor's exposure to these individuals. With that in mind, the proposed alternative model for diagnosing personality disorder is quite beneficial in my setting. A clear and concise way of measuring functionality and traits is less daunting than individually assessing each disorder with the overlapping criteria. And yet, the nature of the disorders in themselves tells me that this new

diagnostic model would be impossible in an outpatient treatment facility. Criteria such as enduring pattern, stable, long-term pattern, treatment resistant, and persistent maladaptive patterns make it clear that these individuals will more than likely find themselves incarcerated and/or institutionalized before they are diagnosed with a personality disorder.

A careful biopsychosocial assessment of individuals suspected of having a personality disorder is a must. In fact, citing the conclusion of this chapter, I cannot imagine diagnosing anyone with any of these disorders without interviewing family, friends, teachers, etc. to gauge the consistency and persistence of the dysfunction. Labeling someone with a personality disorder means more to that person than proper treatment. While it is essential to proper treatment to have an accurate diagnosis, it is also label that follows people into multiple aspects of their lives. Therefore, ensuring that the diagnosis is fitting means the difference between benefit and harm. Just reading one of the sections on special considerations for one of these diagnoses tells me that expertise is indeed needed for fitting diagnosis.

Collaboration and cooperation with other health and mental health professionals is also a strong consideration for me in this area. A second opinion is needed, so to speak, would be my first opinion. Consulting a psychiatrist that will administer psychopharmacological treatment and well as serve as an expert on diagnosis in this area would be my preference. I'm not even sure that I would consider seeing a client who did not have such a resource for his or her care. Tantamount to having a root canal without being on antibiotics for a few days prior, I would wonder if psychotherapy were helpful without a reasonable ability for stable interpersonal relationships. I am generally conservative when it comes to medication, however, when I have a headache, I take something for it. That metaphor is hardly comparable to the unimaginable terror

individuals with personality disorders face daily. What I mean to say is I would never deny anyone the help they needed. This is a dilemma I reacted to strongly from the next chapter.

### **Reflections from Dailey et al., Chapter 17**

My dilemma of being opposed to seeing clients stigmatized and pathologized versus seeing them as having potential to change and heal and altogether wanting to give them the best care possible is echoed strongly at the beginning of this chapter. I do not feel I am alone in seeking out a career in counseling to eliminate stigma and pathology. I do not feel I am alone in feeling that the DSM and its criteria tend to do the opposite. However, I cannot deny the truth that the DSM and the accompanying tools hold great value in treating clients well. To bring the divide between a medical model of treatment and a person-centered model of treatment closer together is going to take some work for me. To not close my mind to the empirical science so closely associated with the DSM is to be a careful scientist myself. What stands out to me about this dilemma is the possibility of holding them both in one hand. Person-centered therapy is not about the absence of scientific knowledge. It is about the relationship stance one takes with clients. The medical model is not intrinsically devoid of empathy and compassion. It is formed from empathy and compassion. If I can keep the data on the pages in front of me close enough to the image of the person in front of me, I will stand a greater chance of integrating the two models.

Advocating for the use of DSM and American Psychological Association (APA) resources has never been on my mind, most likely because of the dilemma mentioned above. I am not settled on how I feel about the challenge to take on the mantle of advocate given by the authors. One part of me feels an appropriate amount of pressure to be a voice in the conversation. Another part of me feels the struggle to integrate my biblical worldview with the new stances on

multi-culturalism (I am not opposed to a multi-cultural view as noted above, I simply chose to communicate it differently). I sense that, if my voice were as accepted as the others currently advocating, I would be more inclined to speak up. I can't say that I feel passionate about becoming a spokesperson for DSM-style assessment and diagnosis. But I do feel passionate about resources that provide the best care possible for human beings. Until we have an organization and manual with a better approach, I will speak (however loudly or softly) for the counseling community's interests. As for the future, the next section will speak to that.

### **Implications for Counseling, Supervision, Counselor Education, Research and Scholarship, and Leadership and Advocacy in the Field of Professional Counseling**

Why is it important to have a manual on mental health nosology? Why do we need to compartmentalize, categorize, and marque people? I wish to focus on three reasons to do so and three reasons our field needs to pay attention to the DSM 5.

The first reason is that our specific nosology needs a common language. Language is a treasure. God only created one species with the ability to form words as a part of communication. Where there is no communication, chaos ensues. Where there is no vision, people perish. The truth is communication about and language about mental health disorders will arise from the utter need to understand. It is incumbent upon those who can codify to codify a methodology of a common language in order to steer the ship of mental health diagnosis together. The ship is too great a burden for one person or even one group of people. When one of us cries for help, the others need to know quickly what is being called for. The common language of the DSM is (even if it is not always adequate) a common language to return to. Emerging counselors, researchers, clinicians, educators, and professional organizations can have extensive conversations around this language. But the language must stay dynamic. If one or more of us digs in on the language

or the mode of communication, we will all go down together. Future researchers, students, and clinicians can rally around the language with the goal of helping more people heal in the next generation, should the Lord wait.

Similar to this reason is the fact that our field of professionals can be a champion in the protection of the weak. I don't mean that it is impossible for those diagnosed with a mental disorder to protect themselves. I mean that we can stand in the gap with them as experts in the field of mental health to ensure they receive the care they deserve. However, there are those, including children and marginalized races and cultures, who are weak simply due to their ability to impact the greater majority. These are the weak that I believe we can help protect. A proper and fitting diagnosis that results from an empirically based assessment can give the weak power. An empowered person can rally a small group of people to do just about anything, including collective healing from trauma and shame. The DSM 5 and the resources that the APA designed can be a source of power to a counselor in an office of one to gain understanding and offer appropriate care.

Lastly, the reason for a unified nosology is that history tends to repeat itself. Teaching the future of psychotherapy that what was damaging in the past will be damaging in the future is paramount. Likewise, teaching them that what was helpful in the past will be helpful in the future is important as well. And yet, handing them a torch to run with versus handing them a dogma to keep safe is my goal. Accept that what was helpful in the past can be improved upon from generation to generation. It is my hope to provide the future with enough confidence to take mental health nosology beyond where it was when they arrived on the scene. Researchers can build upon research and do. Clinicians can build upon techniques and do. The DSM must stay a



working manual for the future of our field to adjust and refine as new evidence and technology arises.

### **Conclusion**

Counseling is not a one-size-fits-all endeavor. Not for the therapist, and not for those seeking therapy. There are too many variables that constantly change in society, human nature, brain science, relationships and interpersonal interaction that keep it from being so. I cannot, therefore, see the DSM 5 or any version thereof as a dogma. Human researchers and human scientists and human writers and editors have had too much influence on this resource for me to do so. I myself am not exempt from failing to be perfect. Because of these facts and more, I am limited to seeing the DSM 5 as a tool for a specific purpose. Albeit a well-crafted, time-tested, research-based tool, it is one tool in my toolbox. It is a tool that I do not know how to use well. It is a tool that I will spend a lifetime learning to use it, when to use it, and for whom to use it.

This reading a reflective assignment has been eye-opening and challenging. It is at once a pressure to persevere and a delight to feel a part of a greater purpose and people. Here's to a lifetime of a lifestyle.

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250/250

This is excellent Ray. Beautiful integration of engaging and meaningful reflection with text content. The time and energy you spent considering the deeply complex themes raised in the texts is apparent. Your score reflects my awareness and valuing of your work.